

DISCHARGE SUMMARY

Discharge Date: June 4, 2026

Vasquez, Maria L.

MRN	RO-221074	DOB	03/22/1948	Age / Sex	78 F / Female
Admitted	June 2, 2026	Discharged	June 4, 2026	Attending	Dr. James T. Calloway, MD — Orthopedic Surgery

DIAGNOSES

Admitting Diagnosis:
Right hip osteoarthritis with functional decline — elective total hip arthroplasty

- Discharge Diagnosis / Diagnoses:**
- Status post right total hip arthroplasty (posterior approach, cementless)
 - Osteoarthritis of the right hip, end-stage
 - Hypertension — perioperatively managed
 - Obesity (BMI 33.2)

HISTORY OF PRESENT ILLNESS

Mrs. Vasquez is a 78-year-old woman with a 3-year history of progressive right hip pain secondary to severe osteoarthritis confirmed on X-ray (Kellgren-Lawrence grade 4). Conservative management with physical therapy, NSAIDs, and two intra-articular corticosteroid injections failed to provide durable relief. She elected to proceed with elective right total hip arthroplasty. Pre-operative workup including CBC, BMP, EKG, and cardiac clearance from her PCP were completed prior to admission. She was admitted on the morning of surgery.

HOSPITAL COURSE

Surgery: Right total hip arthroplasty performed on June 2 under spinal anesthesia with sedation. Posterior approach. Cementless acetabular cup and femoral stem implanted without complication. Estimated blood loss 220 mL. No intraoperative complications.

Post-Operative Recovery: Patient extubated/recovered without difficulty. Pain managed with multimodal analgesia (acetaminophen scheduled, oxycodone PRN, meloxicam held given renal status). Foley removed POD 1. Voiding without difficulty.

Physical Therapy: PT initiated POD 1. Patient ambulated 50 feet with a walker, weight-bearing as tolerated. Hip precautions reviewed and reinforced. Stairs attempted and cleared with supervision POD 2. Discharged to home.

DVT Prophylaxis: Initiated rivaroxaban 10 mg daily POD 1. Will continue for 35 days post-operatively per AAOS guidelines.

Wound: Stapled incision clean, dry, intact at discharge. No signs of infection. Staples to be removed at 14-day wound check.

PAST MEDICAL HISTORY

- Hypertension — on amlodipine
- Hypothyroidism — on levothyroxine
- Obesity (BMI 33.2)
- GERD — on omeprazole
- No prior surgeries on this hip

DISCHARGE MEDICATIONS

MEDICATION / DOSE	ROUTE / FREQ	NOTES
Rivaroxaban 10 mg	PO daily x 35 days	NEW — DVT prophylaxis. Do NOT take NSAIDs concurrently.
Oxycodone 5 mg	PO q6h PRN	For breakthrough pain only. Do not drive while taking. 15-tab supply.
Acetaminophen 650 mg	PO q8h scheduled	Do not exceed 3g/day total including PRN products.
Amlodipine 10 mg	PO daily	Continue home dose.
Levothyroxine 75 mcg	PO daily (fasting)	Continue home dose.
Omeprazole 20 mg	PO daily	Continue.

■ PENDING ITEMS — PCP ACTION REQUIRED

- Rivaroxaban — confirm patient understands full 35-day course. Do not stop early.
- Opioid supply limited to 15 tablets — no refills. Transition to acetaminophen/ice.
- Hip precautions (posterior): no hip flexion >90 degrees, no internal rotation for 6 weeks.

FOLLOW-UP PLAN

SPECIALTY	FOLLOW-UP DATE	INSTRUCTIONS
Orthopedic Surgery	June 16, 2026	Wound check, staple removal, review X-ray, confirm PT progress.
Outpatient PT	June 7, 2026	3x/week hip strengthening and gait training. Prescription provided at discharge.
Primary Care	June 18, 2026	Medication reconciliation, BP check. Discuss restarting meloxicam vs long-term plan.
Orthopedic Surgery — 6-week	July 14, 2026	Full post-op assessment, functional outcome scoring, return to activity clearance.

DISPOSITION & INSTRUCTIONS

Discharged home with her daughter who will assist for the first 2 weeks. Patient instructed in hip precautions, use of assistive devices (walker), incision care, signs of DVT (calf swelling, warmth, pain), and pulmonary embolism (sudden dyspnea, pleuritic chest pain). Return to ED immediately for fever >101.5°F, wound drainage, sudden severe pain, or signs of DVT/PE. Patient verbalized understanding of all instructions. Discharged in stable condition.

Electronically signed by: **James T. Calloway**, MD, FAAOS — Orthopedic Surgery, Riverside Orthopedic & Surgical Center
Date/Time: June 4, 2026 | Riverside Orthopedic & Surgical Center

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